

Community Feedback, Complaints and Response Mechanism (CFCRM)

Professional Guideline and Standard Operating Procedure

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Position	MEAL Coordinator
Document Type	Guideline and Standard Operating Procedure
Applies To	All programmes, projects, field offices, partners, contractors and implementation teams
Primary Areas	Accountability, community engagement, protection, MEAL, programme quality, safeguarding and information management
Review Cycle	At least annually and following major emergencies, operational changes or significant incidents

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1. PURPOSE

The purpose of the Community Feedback, Complaints and Response Mechanism (CFCRM) is to ensure that people affected by crisis can obtain accurate information, ask questions, provide suggestions and feedback, report concerns, raise complaints about services, staff, partners or programme decisions, safely report sensitive protection or safeguarding concerns, receive appropriate responses, influence programme decisions, and see evidence that their feedback contributes to organizational learning and improvement.

Management perspective A CFCRM should not be treated simply as a complaints box or hotline. It is an organizational accountability system connecting communities, programme teams, management, protection/safeguarding structures, MEAL and senior leadership.

2. GUIDING PRINCIPLES

The organization shall operate the mechanism according to the following principles. These principles are operational requirements rather than a checklist: each should be reflected in channel design, case handling, data management and management decisions.

Table: Guiding Principles and Operational Requirements

Principle	Operational requirement
Do No Harm	Feedback collection must not expose people to additional protection risks, retaliation, stigma, discrimination, exploitation, violence, family or community conflict, political consequences, loss of assistance, or privacy risks.
Confidentiality	Information shall only be shared with people who genuinely need it to perform their role. Personal information must not be unnecessarily circulated through informal or public communication channels.
Informed Consent	People should understand why information is being collected, how it will be used, who may access it, whether they can remain anonymous, whether identifying information is optional, and what response they can expect.
Accessibility and Inclusion	Mechanisms must be accessible to women and men, children and adolescents through appropriate safeguarding procedures, older people, people with disabilities, minority groups, people with low literacy, different language groups, displaced people, host communities, people in remote areas, people without phones or internet, and people at heightened protection risk.
Independence and Impartiality	Where a complaint concerns a particular staff member or programme team, the person receiving the complaint should not decide the outcome where there is a conflict of interest.
Timeliness	Every category of feedback should have a defined response timeframe, with more serious or sensitive complaints receiving faster escalation and handling.

Survivor-Centred Approach	For SEA, sexual harassment, abuse, exploitation and serious protection concerns, prioritize safety, confidentiality, informed consent, appropriate referral and avoidance of repeated interviewing or victim-blaming.
Closing the Feedback Loop	The organization should communicate that feedback was received, what action was possible, what action was taken, why action could not be taken where applicable, and what will happen next.
Learning and Continuous Improvement	Feedback must influence programme design and implementation, targeting, communication, staff behaviour, service quality, protection risk mitigation, resource allocation and organizational policies.

3. WHAT THE SYSTEM SHOULD COVER

A mature CFCRM should handle six broad categories. The categories should be visible in the case taxonomy so that routine information requests are not managed in the same way as sensitive complaints.

Table: CFCRM Coverage Categories

Category	Purpose / examples	Management treatment
1. Information Requests	Questions about distribution dates, registration points, required documents or eligibility.	Provide accurate information and record the interaction where required.
2. Assistance / Service Requests	Requests for information about available services, referral information or support within programme scope.	Assess, respond or refer.
3. Suggestions and General Feedback	Recommendations, positive feedback, distribution suggestions and accessibility comments.	Acknowledge, analyse and use for learning.
4. Programme Complaints	Exclusion, targeting, poor-quality assistance, staff behaviour, delays, discrimination, inaccessible services or incorrect information.	Register, review, respond and implement corrective action.
5. Sensitive Complaints	Fraud, corruption, conflict of interest, abuse of power, serious misconduct, threats, exploitation or serious protection concerns.	Restricted handling and referral through the appropriate specialist pathway.
6. Safeguarding / SEA / PSEA	Safeguarding, SEA, sexual harassment, abuse or exploitation allegations.	Do not treat as ordinary programme complaints; activate specialized procedures and referral pathways.

4. DISTINGUISH FEEDBACK FROM COMPLAINTS

Table 1. Feedback and Complaint Types

Type	Meaning	Typical response
Question	Person needs information	Provide information
Request	Person requests assistance/service	Assess and respond/refer

Suggestion	Person recommends improvement	Record, analyse
Feedback	Opinion or experience	Acknowledge, analyse
Complaint	Person alleges failure/problem	Investigate/respond
Sensitive complaint	Potential serious harm/misconduct	Restricted handling/referral
Safeguarding allegation	SEA/SH/abuse/exploitation	Immediate safeguarding pathway
Emergency protection concern	Immediate risk	Immediate protection/referral pathway

Feedback mechanisms and serious complaints mechanisms can overlap, but organizations should maintain an appropriate distinction between routine programme feedback and serious complaints involving misconduct or abuse.

5. SYSTEM DESIGN: FROM ZERO

The mechanism should be formally owned by the organization rather than treated as an informal MEAL activity. Establish the policy, accountability principles, roles and responsibilities, confidentiality requirements, referral pathways, data protection requirements, response standards, escalation procedures, reporting requirements and budget before operational rollout.

Figure: CFCRM establishment pathway

Organizational commitment	Context & risk assessment	Community consultation	Channel design	SOP & roles	Pilot & test	Launch	Monitor & improve
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6. CONDUCT A CONTEXT AND RISK ASSESSMENT

Before selecting channels, the organization should assess the operating environment and identify barriers and risks that could affect access, safety, confidentiality and response. The assessment should cover community characteristics, the protection environment, the communication environment and existing complaint or referral systems.

Table: Context and Risk Assessment Areas

Assessment area	Key issues to assess
Community characteristics	Population size; languages; literacy; gender norms; age distribution; disability; displacement status; geographic distribution; technology access.
Protection environment	Conflict; political sensitivities; GBV risks; child protection risks; retaliation risks; community power structures; discrimination; stigma.
Communication environment	Mobile network; radio; internet; community meetings; religious/community structures; help desks; existing hotlines.
Existing systems	Government complaint systems; interagency mechanisms; cluster mechanisms; protection referral systems; partner mechanisms; community structures.

7. CONSULT COMMUNITIES BEFORE DESIGNING THE MECHANISM

The mechanism should not be designed entirely from the office. Community consultation should establish preferred channels, barriers to complaining, trusted entry points, confidentiality concerns, language requirements, convenient hours and preferred response methods. Separate consultations should be considered for groups that may experience different access or safety barriers.

Table: Community Consultation Framework

Consultation dimension	Questions to explore
Access	How would people prefer to provide feedback? What prevents people from complaining? What hours are convenient?
Trust and safety	Who would people trust? Would people feel safe using a hotline? What would make people afraid to complain?
Gender and inclusion	Would women feel comfortable speaking to male staff? What adaptations are needed for people with disabilities, older people or marginalized groups?
Confidentiality	Are anonymous complaints important? What information should remain confidential?
Response	How should responses be provided? What language and communication method are preferred?

8. SELECT MULTIPLE CHANNELS

A strong mechanism normally uses multiple channels rather than relying on one. The final channel mix should reflect the context assessment and community consultation. Face-to-face, written, telephone, digital, community-based and accessible channels should be considered as complementary options.

Table: Multi-Channel CFCRM Design

Channel family	Examples	Primary value
Face-to-face	Help desks; community centres; trained focal points; programme staff; designated feedback officers.	Human interaction and assisted reporting.
Written	Feedback forms; complaint forms; suggestion boxes; complaint cards.	Useful where written or anonymous submission is preferred.
Telephone	Hotline; call centre; missed-call system; SMS.	Rapid access where mobile connectivity exists.
Digital	WhatsApp; SMS; web form; email; online portal; chatbot where appropriate.	Convenient and scalable digital communication.
Community-based	Community meetings; focus groups; participatory monitoring; home visits; post-distribution monitoring.	Reaches groups through programme and community processes.
Accessible	Sign language; audio; large print; pictorial tools; local languages; assisted reporting.	Reduces communication and disability-related barriers.

9. CHANNEL SELECTION MATRIX

Table 2. Channel Selection Matrix

Channel	Strength	Limitation	Appropriate use
Hotline	Fast/private	Network/phone access	Complaints/questions
Help desk	Human interaction	Privacy concerns	General feedback
Suggestion box	Anonymous	Slow	General feedback
WhatsApp	Convenient	Privacy/data risks	Routine communication
SMS	Accessible	Limited detail	Short feedback
Community meeting	Broad participation	Low confidentiality	General feedback
Household visit	Reaches excluded groups	Resource intensive	Inclusion/monitoring
Online form	Structured	Digital exclusion	Digital populations
PDM	Systematic	Periodic	Programme feedback
Community focal point	Locally accessible	Potential bias/conflict	Routine feedback

Design rule No single channel is universally appropriate. Maintain a combination that balances reach, confidentiality, accessibility, speed and feasibility.

10. CREATE A CLEAR PUBLIC INFORMATION PACKAGE

Communities should receive clear information about who the organization is, what people are entitled to, what they can complain about, how to complain and what happens after a complaint is submitted. The same information should be communicated through channels that are appropriate to the local context.

Table: Public Information Package

Information block	Minimum content
Who are we?	Organization name, programme and implementing partners.
What are people's rights?	Entitlement, eligibility, selection criteria and services available.
What can people complain about?	Programme quality, staff behaviour, targeting, exclusion, delays, discrimination, misconduct and safeguarding issues.
How can they complain?	Hotline, SMS number, office location, opening hours, email, suggestion box and focal point, as applicable.
What happens next?	Receive → register → classify → assess risk → refer/investigate → resolve → communicate → close → analyse → learn.

11. DEVELOP A FEEDBACK AND COMPLAINT SOP

The SOP should translate the policy into a controlled operating process. It should define the purpose, scope, definitions, principles, channels, roles, intake procedure, registration, categorization, prioritization, referral, investigation, response, closure, escalation, confidentiality, data protection, reporting, learning, quality assurance, documentation, retention and disposal.

Figure: SOP operating sequence

Receive	Register	Classify	Prioritize	Refer / Review	Respond	Close	Analyse & Learn
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12. ORGANIZATIONAL STRUCTURE

Figure 1. CFCRM Organizational Accountability Structure

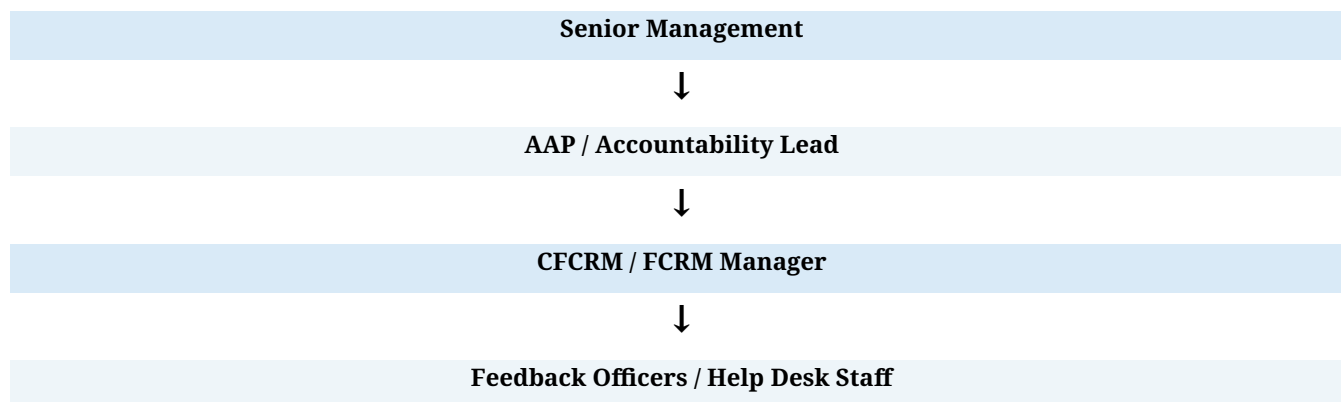


Figure: Cross-functional CFCRM coordination

Programme Teams	MEAL / IM	Protection	Safeguarding / PSEA	HR	Finance / Compliance	Partners
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Each function should have clearly defined responsibilities, referral boundaries and information-access rules.

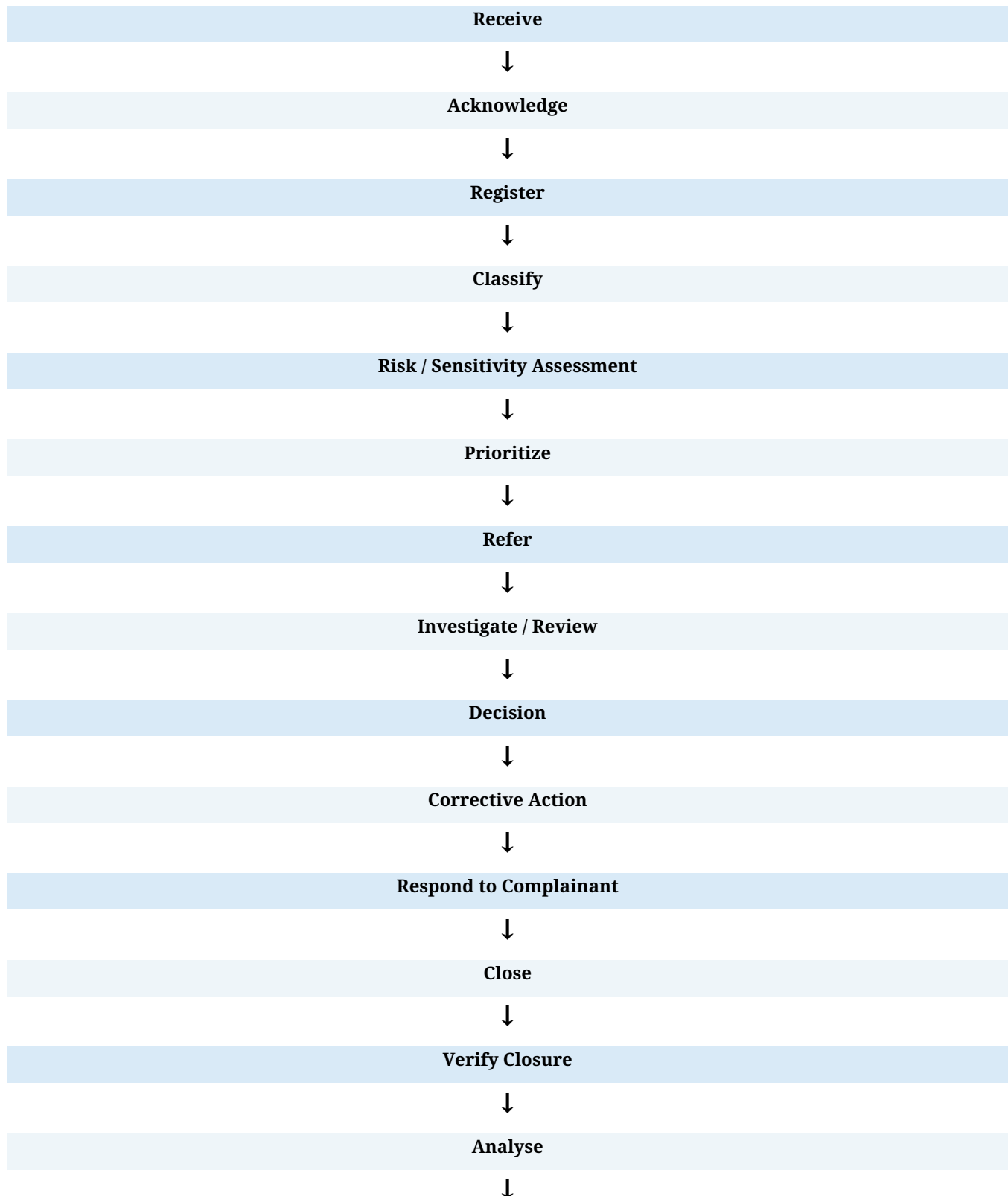
13. KEY ROLES

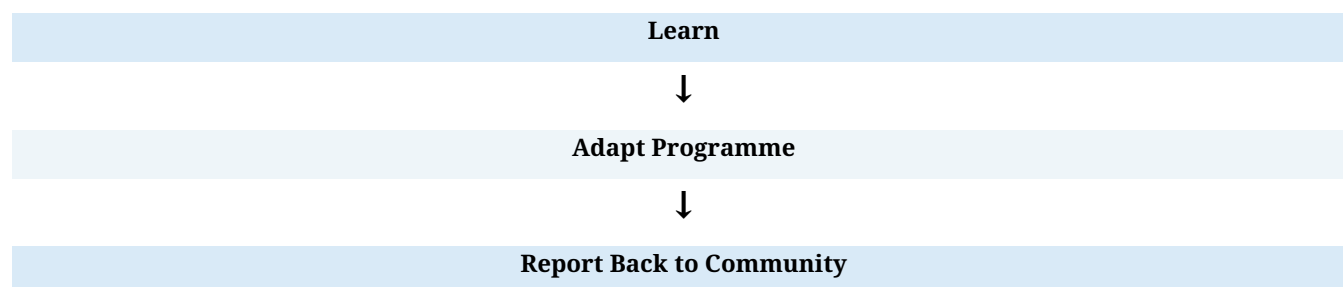
Table: CFCRM Roles and Responsibilities

Function	Core responsibility
Senior Management	Organizational commitment; resources; serious escalations; corrective action; organizational learning.
AAP / CFCRM Manager	Overall mechanism; SOP; staff training; quality assurance; reporting; escalation; coordination.
Feedback Officer	Receive feedback; record cases; classify; acknowledge; refer; follow up; close.
MEAL	Analysis; trends; programme adaptation; monitoring; verification; learning.
Information Management	Database; access control; data quality; dashboards; data security; backups.
Protection	Protection-sensitive cases; referral pathways; risk assessment; survivor-centred handling.
PSEA / Safeguarding	SEA; sexual harassment; safeguarding allegations; specialized referrals; investigation procedures according to organizational policy.
Programme Teams	Respond to programme issues; corrective actions; document actions; communicate results.

14. STANDARD CASE WORKFLOW

Figure 2. Standard Case Management Workflow





Every case should follow a controlled workflow. Sensitive cases should leave the routine workflow at the point of identification and enter the appropriate restricted safeguarding, protection, compliance or investigation pathway.

15. STEP 1: RECEIVING FEEDBACK

When feedback arrives, staff should listen respectfully, avoid arguing or promising a specific outcome, explain the process, assess immediate safety, obtain only necessary information, record the case and provide a reference number where appropriate. A person should never be punished or treated differently because they complained.

16. STEP 2: ACKNOWLEDGEMENT

Suggested acknowledgement “Thank you for contacting us. We have received your feedback/complaint. It has been registered under reference number XXXX. We will review it according to our procedures. If further information is required, we will contact you.”

The acknowledgement should not reveal sensitive information.

17. STEP 3: MINIMUM INFORMATION COLLECTION

Table: Minimum Information Collection

Information group	Recommended fields
Administrative	Case ID; date and time received; location; programme; channel; receiving staff.
Feedback	Category; subcategory; description; requested action.
Demographics	Sex/gender; age group; disability; location; population group, only where justified.
Contact	Preferred contact method; voluntarily provided contact details; preferred language.
Case management	Sensitivity; priority; responsible department; referral; due date; status; action; response; closure date.

18. DO NOT COLLECT UNNECESSARY PERSONAL DATA

The principle is data minimization: collect what is necessary, protect it, use it for the stated purpose and delete it when no longer required. Routine CFCRM intake should avoid unnecessary full family histories, identity numbers, detailed medical or sexual information, photographs and copies of identity documents.

Data minimization test Before adding a field, ask: Is this information necessary for case management, referral, response, safeguarding or learning? If not, do not collect it.

19. CASE CATEGORIZATION

Table: CFCRM Case Taxonomy

Level	Category	Subcategories
A	Information	General information; eligibility; programme schedule; service location.
B	Request	Assistance request; referral request; service request.
C	Feedback	Positive feedback; suggestion; general concern.
D	Programme Complaint	Exclusion; targeting; distribution; service quality; staff conduct; delay; accessibility; discrimination.
E	Serious / Sensitive	Fraud; corruption; conflict of interest; abuse of power; threat; serious misconduct.
F	Safeguarding	SEA; sexual harassment; child safeguarding; exploitation; abuse.

20. PRIORITIZATION

Table: Risk-Based Prioritization

Priority	Typical cases	Management response
1 – Critical	Immediate threat to life; serious safeguarding allegation; SEA; serious child protection concern; immediate retaliation risk.	Immediate escalation according to organizational procedures.
2 – High	Serious misconduct; fraud/corruption; significant protection concern; repeated serious service failure.	Rapid referral and management.
3 – Normal	Programme complaint; targeting concern; staff behaviour; service delay.	Standard response timeframe.
4 – Routine	Suggestion; information request; general comment.	Routine response or next review cycle.

21. RESPONSE TIMEFRAME MATRIX

Table 3. Response Timeframe Matrix

Category	Acknowledge	Action/response target
Critical/safeguarding	Immediate/within 24h	According to safeguarding procedure
High-risk	Within 24h	3–5 working days
Programme complaint	1–2 working days	5–10 working days
Request	1–2 working days	3–5 working days
Suggestion	1–3 working days	Next review cycle
Information request	Same/next working day	1–3 working days

Control point Do not promise a deadline the organization cannot realistically meet.

22. REFERRAL SYSTEM

Table 4. Referral Matrix

Issue	Responsible function	Referral
Programme issue	Programme Manager	Programme team
Protection	Protection	Protection referral
GBV	Specialized GBV service	GBV pathway
Child protection	CP actor	CP pathway
SEA	PSEA/Safeguarding	Specialized pathway
Fraud	Compliance/Management	Investigation
Staff misconduct	HR/Management	HR/investigation
Corruption	Compliance	Investigation
Medical	Health service	Health referral
Legal	Legal/protection actor	Legal referral

The feedback officer should not personally investigate specialized cases unless formally authorized and trained to do so.

23. SENSITIVE COMPLAINT HANDLING

Table: Sensitive Complaint Handling – Do / Do Not

Do	Do not
Listen and ensure immediate safety.	Investigate informally.
Explain confidentiality limits.	Confront the alleged perpetrator.
Respect consent and document minimally.	Promise absolute confidentiality.
Refer appropriately and restrict access.	Circulate case details.
Follow established safeguarding procedures.	Conduct repeated interviews or ask unnecessary questions.
Avoid pressuring the complainant to disclose more.	Pressure the complainant to disclose more.

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24. INVESTIGATION/REVIEW

For ordinary programme complaints, review records, identify the responsible team, verify relevant facts, consult appropriate staff, determine whether an error occurred, identify corrective action and document the decision. Serious allegations should be managed under the organization's specialized investigation procedures.

Figure: Ordinary programme complaint review

Records review	Fact verification	Responsible team consultation	Decision	Corrective action	Documentation
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25. DECISION AND CORRECTIVE ACTION

Table: Corrective Action Options

Possible action	When it may be appropriate
Provide missing information	The issue is caused by an information gap.
Correct registration / reassess eligibility	Administrative or targeting errors are identified.
Replace damaged assistance	Assistance was damaged or otherwise not usable.
Correct or retrain staff	Staff behaviour or capacity contributed to the issue.
Modify distribution / communication	Operational arrangements or messaging created the problem.
Refer / investigate further	The issue requires another service or specialized review.
Change programme design	Feedback identifies a systemic programme issue.
Provide explanation where no change is possible	The requested change cannot reasonably be implemented.

Every action should have a responsible person, deadline, evidence and status.

26. RESPONDING TO THE COMPLAINANT

A response should be respectful, understandable, culturally appropriate, honest, concise and non-defensive. It should explain the outcome without disclosing confidential information about another person.

Response structure We reviewed your complaint concerning [issue]. We found that [brief outcome]. We have taken the following action: [action]. We will [next step] by [date]. Thank you for bringing this issue to our attention.

When the complaint cannot be confirmed We reviewed the issue using the information available to us. We were unable to confirm [issue]. We have nevertheless recorded your concern and will continue monitoring [issue].

27. CLOSING THE CASE

A case should only be closed when the action has been completed, a response has been provided where appropriate, required referrals have been completed, necessary documentation exists and the responsible staff member confirms completion.

Table: Case Status Definitions

Status	Meaning
New	Received but not yet acknowledged or processed.
Acknowledged	Receipt confirmed to the complainant where contact is available.
Under Review	Case is being assessed.
Referred	Transferred to the appropriate function or service.
Pending Information	Further information is needed.
Action Required	Corrective action is pending.
Resolved	Required action completed.
Closed	Case management process completed.
Unable to Contact	Follow-up was not possible after reasonable attempts.
Withdrawn	Complainant withdrew the case.
Rejected / Out of Scope	Outside the mechanism's mandate or not admissible under the procedure.
Escalated	Moved to a higher or specialized level.

28. CLOSURE VERIFICATION

Where feasible, verify whether the complainant received and understood the response, whether the promised action was completed, whether the action created unintended harm and whether further action remains necessary. Satisfaction may be recorded as feedback, but it should not be required as a condition for closing a case.

29. FEEDBACK DATABASE

Table 5. Feedback Database – Minimum Fields

Field	Example
Case ID	FCR-2026-000123
Date received	2026-09-28
Channel	Hotline
Location	Site A
Programme	WASH
Category	Complaint
Subcategory	Service quality

Priority	Medium
Anonymous	Yes/No
Demographic data	Optional
Description	Controlled text
Responsible unit	WASH
Referral	No
Due date	2026-10-05
Status	Under review
Action	Corrective action
Response date	2026-10-03
Closure date	2026-10-03
Outcome	Resolved
Learning theme	Service accessibility

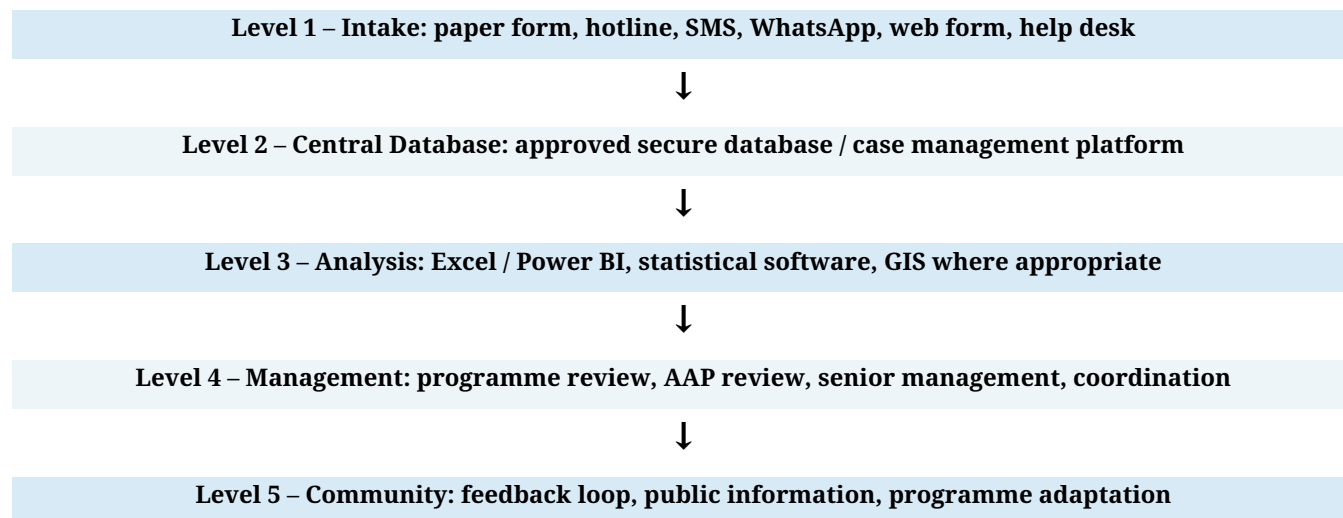
30. UNIQUE CASE IDENTIFICATION

Recommended format ETH-ORO-WASH-2026-000245

In this example, ETH represents the country, ORO the operational area, WASH the programme, 2026 the year and 000245 the sequential case number. Personally identifying information should never be embedded in the case ID.

31. DATA MANAGEMENT ARCHITECTURE

Figure 3. CFCRM Data and Management Architecture



32. DATA PROTECTION CONTROLS

Table: CFCRM Access Control Levels

Access level	Typical users	Access principle
1	Intake staff	Access only to information required for intake.
2	Case managers	Access needed for case management.
3	Programme managers	Access to programme-related information required for action.
4	Protection / safeguarding specialists	Restricted access to sensitive cases.
5	Senior management	Access to management-level information and authorized escalations.
6	Data / IM administrators	Technical access governed by strict controls and audit requirements.

Sensitive case details should have highly restricted access.

33. DATA SECURITY

Digital and physical records should be protected through password protection, role-based access, encryption where available, secure devices, controlled downloads, secure backups, audit logs, approved cloud storage, secure transfer and secure disposal. Sensitive case data should not be retained indefinitely simply because storage is available.

34. PAPER RECORDS

Paper records require the same discipline as digital records. Use locked cabinets, restrict keys, maintain document registers, avoid leaving forms unattended, transfer records securely, establish retention periods and shred or dispose of records securely when authorized.

35. DIGITAL DATA QUALITY

Table: Digital Data Quality Checks

Quality check	Examples
Completeness	Missing fields.
Uniqueness	Duplicate cases or duplicate referrals.
Validity	Invalid dates or impossible ages.
Consistency	Incorrect locations, inconsistent categories or incorrect closure status.
Timeliness	Unresolved cases past deadline.

A database is only useful if the underlying data are reliable.

36. DATA QUALITY RULES

Table: Data Quality Rules

Rule	Application
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Required fields	Fields essential for case management must be completed.
Optional fields	Information that may not be available should remain optional.
Controlled lists	Use predefined values rather than inconsistent free text.
Example	Use one standardized value such as “Female” rather than “Female, woman, F, female person”.

37. DATA ANALYSIS

Table: Feedback Analysis Dimensions

Analysis dimension	What to examine
Volume	Number received, number resolved and number pending.
Category	Questions, requests, suggestions, complaints and sensitive cases.
Programme	WASH, shelter, food, protection, health, cash or other programme areas.
Location	Geographic concentrations and recurring locations.
Demographics	Sex, age, disability and population group where ethically and safely collected.
Channel	Hotline, help desk, community meeting, SMS, digital, suggestion box and other channels.

38. IMPORTANT INDICATORS

Table 6. Key CFCRM Performance Indicators

Indicator	Definition / management question
Accessibility	% of community members who know how to provide feedback.
Awareness	% who know where/how to complain.
Responsiveness	% of cases acknowledged within the required timeframe.
Timeliness	% resolved within the target timeframe.
Closure	% of eligible cases successfully closed.
Referral	% correctly referred.
Inclusion	Feedback participation by relevant demographic groups.
Satisfaction	% satisfied with the feedback process.
Programme adaptation	Number / % of significant programme improvements resulting from feedback.
Trust	% of respondents who believe they can safely complain.

39. CORE DASHBOARD

Illustrative monthly management dashboard

TOTAL FEEDBACK 1,245	COMPLAINTS 310	REQUESTS 420	SUGGESTIONS 180
INFO. REQUESTS 335	RESOLVED 1,080	PENDING 165	OVERDUE 42
AVG. RESPONSE 3.2 days	TOP ISSUE Service accessibility	TOP LOCATION Site B	PROGRAMME WASH
PROGRAMME CHANGES 7			

Table 10. Monthly CFCRM Management Dashboard

Management view	Illustrative result	Decision question
Volume	1,245 total feedback cases	Is volume consistent with community awareness and access?
Resolution	1,080 resolved; 165 pending	What is driving the backlog?
Timeliness	42 overdue; 3.2-day average response	Which categories or teams are exceeding targets?
Issue concentration	Service accessibility	What operational change is required?
Geographic concentration	Site B	Is the issue location-specific?
Programme attention	WASH	What corrective action should programme management take?
Adaptation	7 programme changes	What changed because of community feedback?

Confidentiality rule Sensitive case information should never be displayed on a general management dashboard.

40. TREND ANALYSIS

Trend analysis should move beyond counting complaints. For example, if complaints about distribution delays increase for three consecutive months, management should examine potential contributing factors such as procurement delays, transportation, staffing, communication, targeting and security, and then determine the action required.

Figure: Trend analysis pathway

Signal	Trend	Contributing factors	Management decision	Action	Monitor
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41. ROOT-CAUSE ANALYSIS

For repeated issues, use the Five Whys method to move from the reported symptom to an actionable organizational cause.

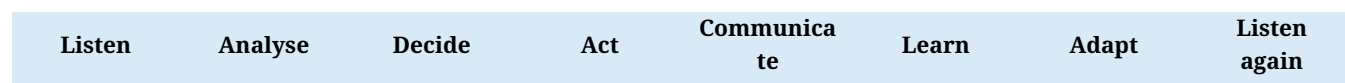
Figure 4. Root-Cause Analysis – Five Whys

Problem: People report repeated delays



42. FEEDBACK TO LEARNING

Figure 5. Feedback-to-Learning Cycle



This cycle is the operational link between community feedback and adaptive programming.

43. PROGRAMME ADAPTATION LOG

Table 7. Programme Adaptation Log

ID	Feedback finding	Decision	Action	Responsible	Deadline	Status
001	Distribution site too far	Change site	Add second site	Programme	15 Oct	Done
002	Information unclear	Improve messaging	New materials	Comms	10 Oct	Done
003	Women uncomfortable reporting	Adjust channel	Female focal point	AAP	20 Oct	Pending

The adaptation log demonstrates whether feedback is producing concrete changes in programming.

44. MONTHLY REVIEW MEETING

Every month, conduct a CFCRM review involving AAP, MEAL, programme, protection, safeguarding/PSEA, operations, communications and senior management as appropriate.

Table: Monthly CFCRM Review Agenda

Agenda area	Review question
Previous actions	What actions remain open and why?
New feedback	What are communities telling us now?
Trends	Which issues are increasing, decreasing or recurring?
Overdue cases	Where are response targets being missed?
Serious risks	Are there protection, safeguarding, fraud or retaliation concerns?
Inclusion	Which demographic or geographic groups are underrepresented?
Programme implications	What changes are required?
Decisions and ownership	Who will do what, by when?

45. QUARTERLY REFLECTION

Table: Quarterly AAP Reflection

Reflection dimension	Key question
Relevance	Are we listening to the issues communities actually care about?
Accessibility	Who is using the system, and who is not?
Safety	Can people complain without fear?
Responsiveness	Are we resolving issues?
Inclusion	Are marginalized groups represented?
Trust	Do people believe complaints are taken seriously?
Learning	What has changed because of feedback?

46. COMMUNITY REPORT-BACK

Table: “You Told Us – We Changed” Report-Back

Community said	Organization response
“The distribution location is difficult to reach.”	“From October, distributions will take place at two locations.”
“Information about eligibility was unclear.”	“We revised the eligibility information and translated it into local languages.”

The organization should periodically communicate what communities said and what changed. Where no change is possible, the organization should explain why.

47. WHEN THE ORGANIZATION CANNOT ACT

Sometimes feedback cannot be implemented. In such cases, explain what was requested, what was reviewed, why it cannot be implemented and what alternative is available. The organization should never create false expectations.

48. STAFF TRAINING

Table: CFCRM Staff Training Framework

Training area	Expected staff capability
CFCRM and AAP	Understand the purpose and accountability role of the mechanism.
Communication and active listening	Receive concerns respectfully and explain the process clearly.
Confidentiality and informed consent	Protect information and explain appropriate use.
Safeguarding, PSEA and protection	Recognize sensitive cases and follow referral procedures.
Data protection	Collect, store, share and dispose of information safely.
Case management	Classify, prioritize, document, escalate and close cases correctly.

49. ROLE-PLAY TRAINING

Table: CFCRM Role-Play Scenarios

Scenario	Staff practice focus
Exclusion from cash assistance	Listening; recording; verification; response.
Inappropriate behaviour by staff	Safe reception; confidentiality; referral; avoiding investigation.
Anonymous fraud report	Recording; confidentiality; compliance referral.
Wrong type of assistance	Categorization; programme referral; learning.

50. COMMUNITY AWARENESS PLAN

Table: Community Awareness Channels

Communication method	Purpose
Posters / leaflets	Visible information about rights and channels.
Community meetings	Explain the mechanism and answer questions.
Radio / SMS	Reach wider or remote populations.
Social media where appropriate	Digital awareness and reminders.
Outreach workers / information desks	Direct support and assisted reporting.
Verbal announcements	Reach people with low literacy.

Figure: Core community awareness message

WHO	WHAT	WHERE	HOW	WHEN	WHAT HAPPENS
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51. ANONYMITY

Anonymous feedback should be allowed where feasible, particularly where people fear retaliation, stigma, loss of assistance, discrimination, conflict with authorities or conflict with community leaders. Anonymous cases may be more difficult to follow up.

Suggested message “You may provide feedback anonymously. If you provide contact information, we may be able to follow up with you.”

52. COMMUNITY FOCAL POINTS

Community representatives can help people access the mechanism but should not control it. Risks include elite capture, confidentiality problems, favoritism, retaliation and filtering of complaints. Community structures should therefore complement, not replace, independent organizational channels.

53. PARTNER MANAGEMENT

Table: Partner CFCRM Requirements

Partner agreement area	Minimum expectation
Standards	Minimum CFCRM and accountability standards.
Reporting	Defined reporting requirements.
Referral	Agreed referral procedures.
Data protection	Agreed data handling and access requirements.
Response	Defined response times.
Escalation	Clear escalation arrangements.
Serious complaints	Defined handling and referral.
Learning and monitoring	Joint analysis, monitoring and audit rights.

The organization should not assume that a partner's informal complaint process meets humanitarian accountability requirements.

54. THIRD-PARTY CONTRACTORS

Contracts with transporters, distributors, call centres, suppliers and other service providers should specify behavioural standards and complaint referral requirements. People should be able to report concerns about contracted personnel as well.

55. INTERAGENCY COORDINATION

Where multiple organizations operate in the same location, coordinated mechanisms may simplify access for communities, reduce the number of hotline contacts, support a common taxonomy, improve referral, reduce duplication and strengthen analysis.

Figure: Interagency coordination pathway

Common access	Common taxonomy	Coordinated referral	Shared learning
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56. COMMON FEEDBACK TAXONOMY

Table: Common Feedback Taxonomy

Code	Category
1	Information Request
2	Assistance Request
3	Suggestion
4	Positive Feedback
5	Programme Complaint
6	Staff Conduct
7	Fraud / Corruption
8	Protection
9	SEA / PSEA
10	Child Safeguarding
11	Other Sensitive
12	Referral

A common taxonomy supports consistent recording and interagency analysis.

57. QUALITY ASSURANCE

Table: CFCRM Quality Assurance Checklist

QA domain	Verification question
Awareness	Are communities aware of the mechanism?
Channel functionality	Are channels functioning?
Registration	Are cases registered correctly?
Timeliness	Are deadlines met?
Protection	Are sensitive cases protected?
Referral	Are referrals working?
Documentation	Are responses documented?
Closure	Are cases being closed correctly?
Learning	Are programme teams using findings and are communities

	seeing changes?
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58. FEEDBACK MECHANISM PERFORMANCE SCORECARD

Table: CFCRM Performance Scorecard

Dimension	Management test
Access	Can people use it?
Safety	Can they use it safely?
Responsiveness	Do they receive responses?
Quality	Are responses appropriate?
Trust	Do people trust the mechanism?
Inclusion	Can marginalized groups access it?
Learning	Does it change programming?
Accountability	Does management act on findings?

59. RED FLAGS

Table: CFCRM Red-Flag Monitoring

Red flag	Possible interpretation / management concern
Sudden decline in complaints	Possible access, awareness, trust or reporting problem.
Unusually high satisfaction	Check whether feedback collection is reaching critical voices.
Zero complaints in a high-risk programme	Do not assume good performance; assess access and fear barriers.
Repeated anonymous allegations	Assess protection, confidentiality and underlying issue.
Many complaints from one location	Investigate geographic concentration.
Repeated complaints against one staff member	Review conduct and management controls.
Increasing unresolved or overdue cases	Assess capacity and response bottlenecks.
People refusing to provide feedback	Assess trust and perceived retaliation risks.
Major differences between community groups	Assess inclusion and access barriers.

A low number of complaints does not automatically mean good programme performance; it may indicate lack of awareness, inaccessible channels, fear, distrust or social barriers.

60. WHAT NOT TO DO

Table: Practices to Avoid

Weak practice	Why it is a problem
Suggestion box only	Excludes people who cannot write or safely access the location.
Hotline only	Excludes people without phones, network access or confidence to call.

Community leader only	Can compromise confidentiality and inclusion.
Treat complaints as negative	Misses an important accountability and learning resource.
Collect everything	Creates unnecessary privacy risks.
Close a case after referral	Referral is not necessarily resolution.
Only report complaint numbers	Numbers without analysis do not demonstrate accountability.
Ask communities but never change anything	Creates consultation fatigue and undermines trust.

61. MINIMUM DOCUMENTATION PACKAGE

Table: Minimum CFCRM Documentation Package

Core document	Purpose
CFCRM Policy	Defines organizational commitment and accountability requirements.
CFCRM SOP	Defines operational procedures and workflows.
Channel Mapping	Documents available feedback channels.
Referral Directory	Provides referral contacts and pathways.
Feedback / Complaint Form	Standardizes intake.
Case Register	Tracks case management.
Sensitive Case Procedure	Controls specialized cases.
Data Protection Protocol	Defines information protection requirements.
Staff Code of Conduct	Sets behavioural standards.
PSEA / Safeguarding Referral Procedure	Defines specialized pathways.
Response Time Matrix	Defines expected timelines.
Escalation Matrix	Defines management escalation.
Monthly Dashboard	Supports management monitoring.
Programme Adaptation Log	Tracks changes resulting from feedback.
Community Report-Back Template	Supports closing the feedback loop.
Training Package	Builds staff capacity.
Monitoring Checklist	Supports quality assurance.
Quarterly Review Template	Supports AAP reflection.
Annual CFCRM Assessment	Supports periodic system review.

62. MASTER CASE REGISTER

Table 8. Master Case Register Fields

Field	Field	Field	Field
Case_ID	Date_Received	Time_Received	Country
Region	District	Site	Programme
Partner	Channel	Feedback_Type	Category
Subcategory	Priority	Anonymous_YN	Sex
Age_Group	Disability_YN	Population_Group	Preferred_Language

Contact_Method	Description	Immediate_Risk	Referral_Required
Referral_To	Referral_Date	Responsible_Unit	Responsible_Person
Due_Date	Status	Investigation_Required	Action_Required
Action_Taken	Response_Date	Response_Method	Closure_Date
Outcome	Complainant_Contacted	Closure_Verified	Learning_Theme
Programme_Change	Data_Quality_Checked	Case_Closed_By	

Sensitive cases should be managed in appropriately restricted systems rather than exposing detailed allegations in a general programme database.

63. CASE MANAGEMENT FORM

Table 9. Case Management Form

Section	Fields
A. Intake	Case ID; date; channel; location; programme.
B. Person	Anonymous?; contact method; optional demographics.
C. Issue	Category; subcategory; description; requested outcome.
D. Risk	Immediate danger?; protection concern?; safeguarding?; confidential?; referral required?
E. Action	Responsible unit; referral; due date; action.
F. Response	Date; method; response provided; further action.
G. Closure	Outcome; closure date; verified?; learning identified?

64. MONTHLY REPORT TEMPLATE

Table: Monthly CFCRM Report Structure

Report section	Required content
1. Executive Summary	Total cases; major trends; significant risks; major actions.
2. Feedback Volume	By programme, location, channel and category.
3. Response Performance	Acknowledgement rate; response rate; closure rate; overdue cases.
4. Inclusion	Analysis of available demographic data.
5. Serious Issues	Appropriately aggregated information only.
6. Programme Learning	Top five issues.
7. Programme Changes	What changed?
8. Management Actions	Who must do what?
9. Community Communication	How were findings reported back?

65. QUARTERLY AAP REFLECTION REPORT

Table: Quarterly AAP Reflection Report

Reflection question	Response field
What did communities tell us?	Document major themes.
What did we do?	Document management and programme actions.
What changed?	Document measurable or observable changes.
What did not change? Why?	Explain constraints and decisions.
Which groups are underrepresented?	Identify participation gaps.
What risks emerged?	Identify protection, safeguarding or operational risks.
What should management change?	Management action.
What should programmes change?	Programme action.
What should staff change?	Staff practice/capacity action.
What should the organization stop, start and continue?	Organizational learning.

66. ANNUAL REVIEW

Table: Annual CFCRM Review Framework

Review area	Assessment focus
Policy and SOP	Are documents current and operational?
Accessibility and channels	Can different groups use the system?
Staff capacity	Are staff trained and competent?
Data protection	Are access, security and retention controls functioning?
Case management and referral	Are cases handled and referred correctly?
Response times	Are standards being met?
Trust and inclusion	Do communities trust the mechanism and are groups represented?
Learning and adaptation	Does feedback influence programming?
Partner compliance	Are partners meeting agreed requirements?

Communities should be consulted directly as part of the annual review.

67. MATURITY MODEL

Table 11. Maturity Model

Level	Maturity stage	Characteristics
1	Basic	Complaint box; informal feedback; little documentation.
2	Structured	Multiple channels; database; SOP; defined roles.
3	Managed	Standardized taxonomy; referral system; dashboards; response monitoring; data

		protection.
4	Integrated	Feedback informs programme decisions; senior management reviews trends; systematic report-back; interdepartmental learning.
5	Adaptive	Communities influence programme decisions; real-time feedback; strong collective mechanisms; advanced analysis; continuous learning; measurable programme adaptation.

68. THE COMPLETE ACCOUNTABILITY CYCLE

Figure 6. Complete Accountability Cycle

Infor m	Cons ult	Liste n	Recei ve	Prote ct	Resp ond	Refer	Resol ve	Close the loop	Analy se	Refle ct	Adap t	Repo rt back	Repe at
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A high-quality organization should ultimately operate this cycle continuously. The objective is not merely to collect feedback, but to connect community voice with safe access, response, learning, management decisions, programme adaptation and report-back.

69. GOLDEN RULES FOR STAFF

Table: Golden Rules for Staff

#	Rule
1	Listen before responding.
2	Never punish someone for complaining.
3	Never promise an outcome you cannot guarantee.
4	Collect only necessary information.
5	Protect confidentiality.
6	Do not investigate sensitive cases informally.
7	Refer specialized cases correctly.
8	Respect the complainant.
9	Document accurately.
10	Respond within the established timeframe.
11	Escalate risks immediately.
12	Use feedback to improve programmes.

70. ORGANIZATIONAL SUCCESS TEST

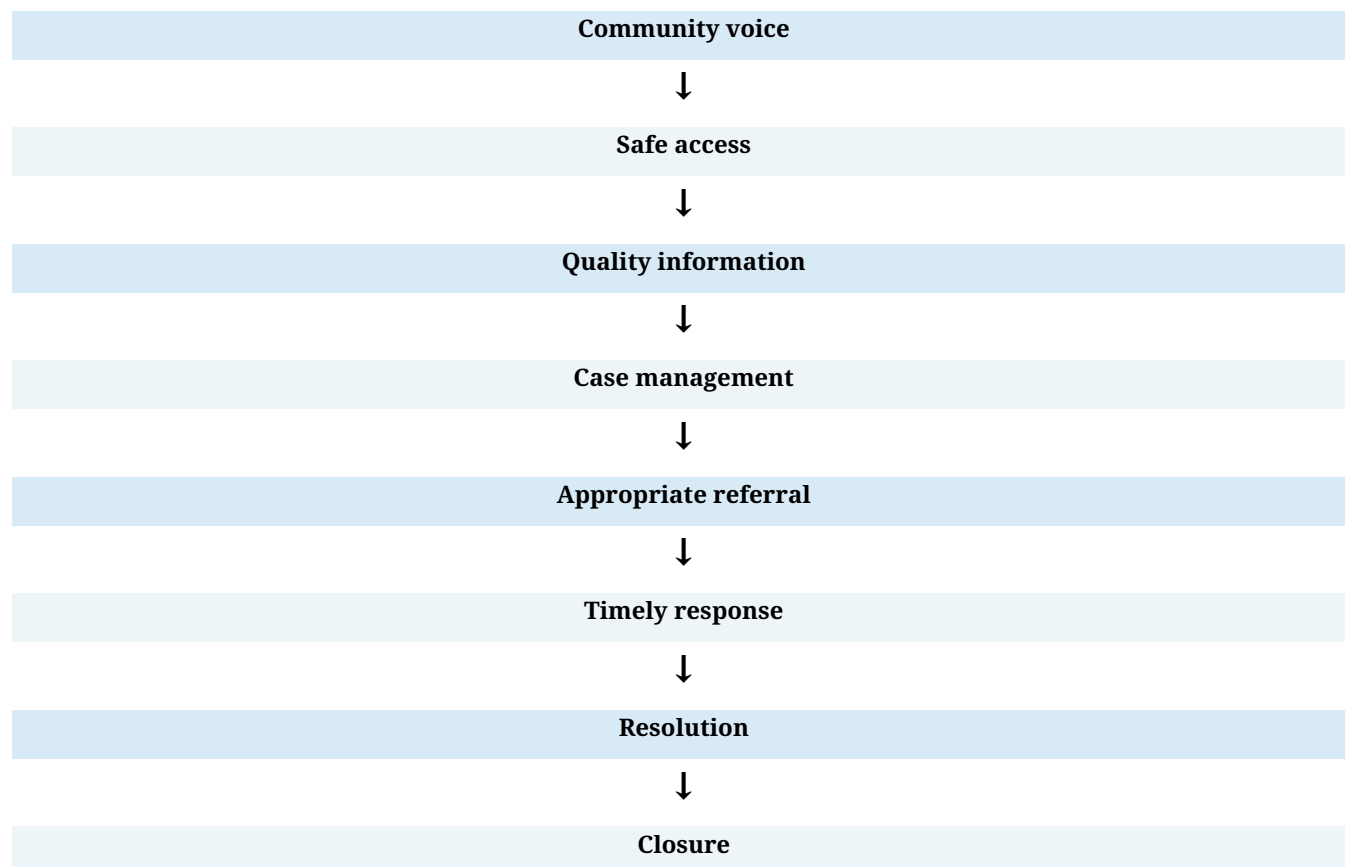
Table 12. Organizational Success Test

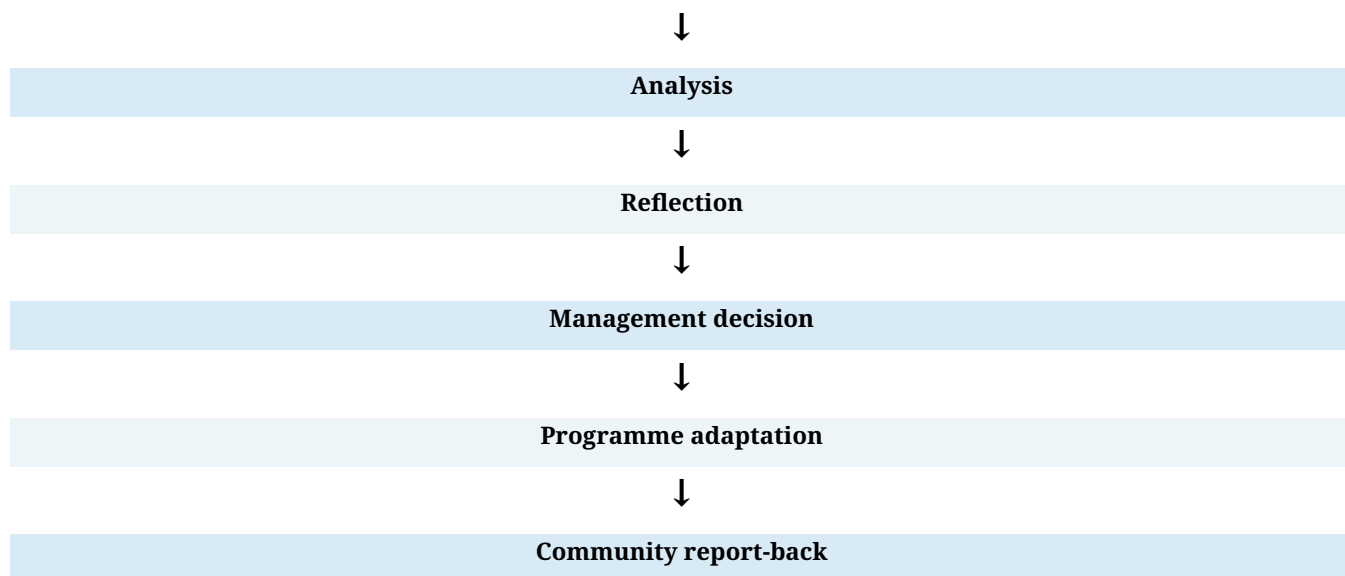
Success indicator	Can affected people answer “yes”?
Awareness	I know how to provide feedback.
Language access	I can complain in my own language.
Choice	I can use more than one channel.
Anonymity	I can complain anonymously if necessary.
Safety	I feel safe complaining.
Process clarity	I know what will happen after I complain.
Response	I receive a response.
Data protection	My information is protected.
Non-retaliation	I am not punished for complaining.
Referral	Serious concerns are referred appropriately.
Learning	I can see that the organization learns from feedback.
Adaptation	I can see changes resulting from community input.

71. FINAL MANAGEMENT PRINCIPLE

Management principle A mature humanitarian feedback mechanism is not a database, hotline, suggestion box or MEAL indicator. It is an organizational accountability system.

Figure: From community voice to organizational accountability





If the process stops at “we collected feedback,” the organization has collected information. If it reaches “we changed our action because people told us something important,” the organization is demonstrating accountability.